

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Brief Study Title: A Study of Elranatamab (PF-06863135) in Chinese Participants With Refractory Multiple Myeloma **Full/Official Study Title:** A Phase 1B/2, Open-Label Study To Evaluate The Safety, Pharmacokinetics, Pharmacodynamics, And Efficacy Of Elranatamab (PF-06863135) In Chinese Participants With Multiple Myeloma Who Are Refractory To At Least One Proteasome Inhibitor, One Immunomodulatory Drug And One Anti-Cd38 Antibody (Triple-class Refractory MM) **Short Title/Acronym:** MagnetisMM-8 **Protocol Number:** C1071008 **NCT Number:** NCT05228470 **Posting ID:** 1063046164827300209 **Sponsor/Company Name:** Pfizer **Investigational Product:** Elranatamab (PF-06863135) / Trade Name: Elrexfio **Phase of Development:** Phase 2 (Phase 1b/2) **Trial Status:** Active, Not Recruiting **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: * **Phase 1b:** To establish the safety profile and confirm the monotherapy Recommended Phase 2 Dose (RP2D) in Chinese participants by monitoring Dose-Limiting Toxicities (DLTs). * **Phase 2:** To evaluate the efficacy of elranatamab monotherapy as measured by the Objective Response Rate (ORR).

Secondary Objectives: To evaluate Time to Response (TTR), Duration of Response (DoR), Complete Response Rate (CRR), Overall Survival (OS), Minimal Residual Disease (MRD) negativity rate, Pharmacokinetics (PK), and Pharmacodynamics (PD).

2.2 Background & Rationale To address the urgent unmet medical need in patients with triple-class refractory multiple myeloma (refractory to at least one proteasome inhibitor, one immunomodulatory drug, and one anti-CD38 antibody). Elranatamab is a bispecific antibody that bridges BCMA-expressing myeloma cells and CD3-expressing T-cells to cause targeted T-cell-mediated cytotoxicity. This study assesses its efficacy and safety profile specifically within the Chinese patient population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target **N**: 39 Participants
Number of Sites: Multiple sites across China **Estimated Study Start:** 21-Dec-2021 (Actual) **Estimated Primary Completion:** 06-Aug-2023 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosis of multiple myeloma (per IMWG criteria, Rajkumar et al, 2014) with measurable disease, defined by at least 1 of the following: Serum M-protein ≥ 0.5 g/dL, Urinary M-protein excretion ≥ 200 mg/24 hours, or Serum immunoglobulin FLC ≥ 10 mg/dL (≥ 100 mg/L) AND abnormal serum immunoglobulin kappa to lambda FLC ratio. 2. Refractory to at least one immunomodulatory drug (IMiD), one proteasome inhibitor (PI), and one anti-CD38 antibody. 3. Relapsed/refractory to last anti-myeloma regimen. 4. ECOG performance status ≤ 2 . 5. Adequate bone marrow function defined by Absolute Neutrophil Count (ANC) $\geq 1.0 \times 10^9/L$, Platelets $\geq 25 \times 10^9/L$, and Hemoglobin ≥ 8 g/dL. 6. Resolved acute effects of any prior therapy to baseline severity or CTCAE Grade ≤ 1 . 7. Not pregnant and willing to use contraception.

4.2 Exclusion Criteria 1. Diagnosis of smoldering multiple myeloma, active plasma cell leukemia, amyloidosis, or POEMS syndrome. 2. Stem cell transplant or active Graft-Versus-Host Disease (GVHD) within 12 weeks prior to enrollment. 3. Previous treatment with an anti-BCMA directed therapy. 4. Impaired cardiovascular function or clinically significant cardiovascular diseases. 5. Ongoing Grade ≥ 2 peripheral sensory or motor neuropathy, history of Guillain-Barré syndrome (GBS) or GBS variants, or history of any Grade ≥ 3 peripheral motor polyneuropathy. 6. Active HBV, HCV, SARS-CoV2, HIV, or any active, uncontrolled bacterial, fungal, or viral infection. 7. Any other active malignancy within 3 years prior to enrollment, except for adequately treated basal cell or squamous cell skin cancer, or carcinoma in situ. 8. Previous administration with an investigational drug within 30 days or 5 half-lives preceding the first dose of study intervention used in this study (whichever is longer).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Elranatamab administered in 2 step-up priming doses followed by full therapeutic monotherapy doses. **Route of Administration:** Subcutaneous (SC) injection. **Duration of Treatment:** Expected participation and follow-up for approximately 2 years.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease-related symptoms and

prophylaxis for infections. **Prohibited:** Previous administration of an investigational drug within 30 days or 5 half-lives preceding the first dose. Other concurrent anti-cancer therapies are strictly forbidden.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, BM Assessment, Baseline Labs
Baseline	Day 0	First Dose (Priming), PK/PD sampling, Safety Observation
Interim	Every 4 Weeks	Safety Labs, Efficacy Assessment (IMWG criteria), Subcutaneous injections
End of Study	Up to 2 years	Final Efficacy Assessment, Survival Follow-up, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- * **Phase 1b:** Number of Participants With Dose-Limiting Toxicities (DLTs) up to 4 weeks. (Includes parameters such as Grade 4 neutropenia >5 days, febrile neutropenia, Grade 4 thrombocytopenia, Grade ≥ 3 CRS not resolving to $\leq$ Grade 1 within 48h, etc.).
- * **Phase 2:** Objective Response Rate (ORR) by Blinded Independent Central Review (BICR) as Per International Myeloma Working Group (IMWG) Criteria. Evaluates the percentage of participants with best overall response of confirmed stringent complete response (sCR), CR, very good partial response (VGPR), or PR.

7.2 Secondary & Exploratory Endpoints

- * Duration of Response (DOR) as Per IMWG Criteria by BICR
- * Duration of Response (DOR) as Per IMWG Criteria by Investigator Assessment
- * Complete Response Rate (CRR) as Per IMWG Criteria by BICR
- * Time to Response (TTR)
- * Minimal Residual Disease (MRD) negativity rate
- * Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection of AEs with specific protocols for monitoring immune-related toxicities, including Cytokine Release Syndrome (CRS) and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). **Serious Adverse Events (SAEs):** Reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal safety review committee utilized to clear dose-escalation phases and establish the RP2D.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all efficacy outcomes; Safety Set for all participants receiving at least one dose of the study drug. **Sample Size Justification:** Utilizes a single-stage design for the Phase 2 part adequately powered to evaluate the efficacy (ORR) and safety profile in the Chinese cohort. **Handling of Missing Data:** Missing response data handled by treating patients as non-responders for categorical endpoints; standard censoring for time-to-event endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** High-frequency onsite and remote monitoring tailored for early phase oncology studies evaluating bispecific antibodies. **Audit Trail:** Robust eCRF locking mechanisms, safety data reconciliation, and data clarification forms.

11. Study Identifiers & Governance

Primary ID: C1071008 **Registry Number:** NCT05228470 **Posting ID:** 1063046164827300209 **Sponsor/Lead Organization:** Pfizer **Study Title:** MagnetisMM-8 **Source Level:** 5 **MeSH Code:** D009101 **HPO Code:** HP:0006775 **SNOMED IDs:** 109989006, 269475001, 415111003, 1162576007, 64572001 **EMA Product Information URL:** [Elrexio EPAR](#) **Disease Areas/Conditions Addressed:** Elranatamab, Myeloma, Multiple Myeloma, Relapsed Multiple Myeloma, Refractory Multiple Myeloma, PF-06863135, BCMA, Bispecific, Bispecific Antibody, BCMA-CD3 Bispecific, MagnetisMM-8

12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Refractory Multiple Myeloma (Preferred UMLS Name: Refractory Multiple Myeloma) **Semantic Type:** Neoplastic Process **Diagnosis Threshold:** Triple-class refractory (IMiD, PI, and anti-CD38 mAb exposed) **Medical Methodology:** Bispecific antibody T-cell engager **Optimization Target:** CD3-directed T-cell-mediated cytotoxicity against BCMA-expressing myeloma cells

13. Study Procedures & Methodology

Management Pathway: Step-up dosing mechanism to mitigate the severity of Cytokine Release Syndrome (CRS). **Education Protocol (HCP):** Aggressive CRS and ICANS management, including the use of tocilizumab and corticosteroids. **Education Protocol (Patient):** Symptom awareness reporting for immediate hospital admission

during priming steps. **Quality Audit Mechanism:** Central lab evaluations for M-protein and Free Light Chains (FLC); blinded independent central review (BICR) for response confirmations.

14. Observation & Follow-Up Schedule

Total Duration: ~2 years follow-up **Onsite Visit Cadence:** Weekly during initial cycles (step-up dosing), scaling back to Day 1 of subsequent 28-day cycles **Remote Monitoring Frequency:** Intermittent telehealth and safety follow-ups as required **Checklist Review Interval:** Continuous every cycle (28 days)

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]					
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]					